

Hepatitis C Framework

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Table of contents

Table of contents.....	2
Purpose	3
Hepatitis C in Correctional Health Services.....	3
What is Hepatitis C?.....	3
Screening for Hepatitis C	4
Referral Information.....	4
Community Referrals	5
Rural/ Remote Referrals	Error! Bookmark not defined.
Hepatitis C Treatment Education	5
Data Collection	6
Audits and Sustainability	6
Contributors.....	6
Contact.....	6

Purpose

To provide background information and a standardized process for identifying, diagnosing, and treating Hepatitis C in Provincial Corrections.

Hepatitis C in Correctional Health Services

Hepatitis C (HCV) can be up to 40x more prevalent in justice involved individuals. Provincial correctional centres have been identified as the number one priority for elimination of Hepatitis C by 2030 in Canada¹.

Historically, treating HCV for incarcerated patients has been difficult due to barriers such as access and limited resources. A recent Hepatitis C pilot project in three Calgary Correctional Centres demonstrated meeting with patients to discuss HCV treatment and education was a necessary first step in HCV elimination.

There are two ways patients can be treated in corrections:

- The Corrections Formulary (AHS)
 - Patients must have no pending court dates for at least 24 weeks
- Community Partners
 - Patients can be referred to community partners for treatment plans pre and post release based on a number of factors

See Appendix A for more details.

What is Hepatitis C?

Hepatitis C (HCV) is an infection of the liver caused by the Hepatitis C virus. It is transmitted through blood contact of an infected individual.

HCV is most commonly spread through:

- sharing equipment for drug use (injection, inhalation, and snorting)
- re-using tattoo needles
- sharing personal hygiene equipment (razors, toothbrushes)
- unprotected sex (less common)

¹ Action Hepatitis Canada (2022) *Prison Health is Public Health: The right to Hepatitis C prevention, diagnosis and care in Canada's correctional settings*.
[https://www.actionhepatitiscanada.ca/prisonhealth.html#:~:text=People%20who%20are%20incarcerated%20\(PWAI,health%20care%20in%20the%20community](https://www.actionhepatitiscanada.ca/prisonhealth.html#:~:text=People%20who%20are%20incarcerated%20(PWAI,health%20care%20in%20the%20community).

HCV often has no symptoms, which means getting tested is the best way to identify infection in a high-risk population. Untreated HCV will cause liver damage over time which can result in liver scarring, cirrhosis, and failure.

Screening for Hepatitis C

There are two blood tests needed to identify a current HCV infection:

- HCV antibody indicates if a patient **has ever** been exposed to Hepatitis C
- HCV RNA indicates if the patient **currently** has Hepatitis C

On admission, the healthcare professional shall:

- Offer STBBI testing which includes HCV testing
- Ask the patient if they have a known history of HCV
 - If yes, the healthcare professional shall contact an authorized prescriber to order the HCVRNA serology on the admission order set.
 - Note: When ordering HCVRNA, ensure the lab requisition indicates the specimen is from a Correctional Centre. This will mitigate long wait times for lab results.

Additional Screening

Sometimes, patients don't know that they have a history of HCV. The assigned healthcare professional at each site shall have a process for case finding which includes:

- Reviewing Netcare history
- Offering follow up bloodwork as indicated

Referral Information

Once bloodwork has been completed, a healthcare provider shall:

- Meet with the patient to provide education (see Appendix B)
- Refer the patient for treatment as indicated
 - Refer to Centre Prescriber if patient will be in corrections for at least 24 weeks
 - Refer to Community Partner if patient will be in corrections for less than 24 weeks

Note: A patient who is HCV antibody positive, but HCV RNA negative does **not** currently have Hep C. A patient who is HCV antibody positive **and** HCV RNA positive has a current Hep C infection and needs education regarding treatment options.

The healthcare professional shall consider the following when determining a treatment care plan for a patient with HCV:

- Have they been treated before?
- When is their next court date/ release date?
- Does the patient give informed consent to start treatment?
- Has the patient ever been connected in the community for HCV treatment?
- Does the patient have any kind of drug coverage in the community? (NIHB, AISH, etc)

See Appendix C for further rationale to each of the above questions

Community Referrals

Each site has different community partners for patients who require HCV treatment and follow up in the community. Some of these community partners can start patients on treatment while they are still in centre. The healthcare professional shall liaise with the community partner to come up with an individualized care plan for the patient. This includes:

- Scheduling follow up appointments in the community
- Sending all remaining HCV medication on release with the patient
- Providing community resources and supports as required

See Appendix D for some established community partners.

*Note sites do not need to use a community partner that is listed.

Hepatitis C Treatment Education

HCV is treated using direct acting antivirals (DAAs) such as Epclusa or Maviret. The medication comes in pill form and is taken once daily for either 8 weeks (Maviret) or 12 weeks (Epclusa). Treatment has a ~95% efficacy when taken as prescribed and there are little to no side effects. About 20% of patients with HCV may naturally clear the infection.

The designated healthcare provider shall provide the following education to patients with HCV:

- Overview of HCV and importance of treatment
- Patient specific risk factors
- How to take the medication
- Risk of re-infection

- Post treatment bloodwork

Data Collection

All HCV activities are captured in FSIS per site processes.

Audits and Sustainability

This framework will be audited 4 months after launch. The audit results shall be shared with the quality council and actioned appropriately.

Contributors

The following people have contributed to this framework:

Kayley Goralczyk, Provincial Assistant Head Nurse, Correctional Health Services

Jordana Schmitz, Nursing Practice Lead, Correctional Health Services

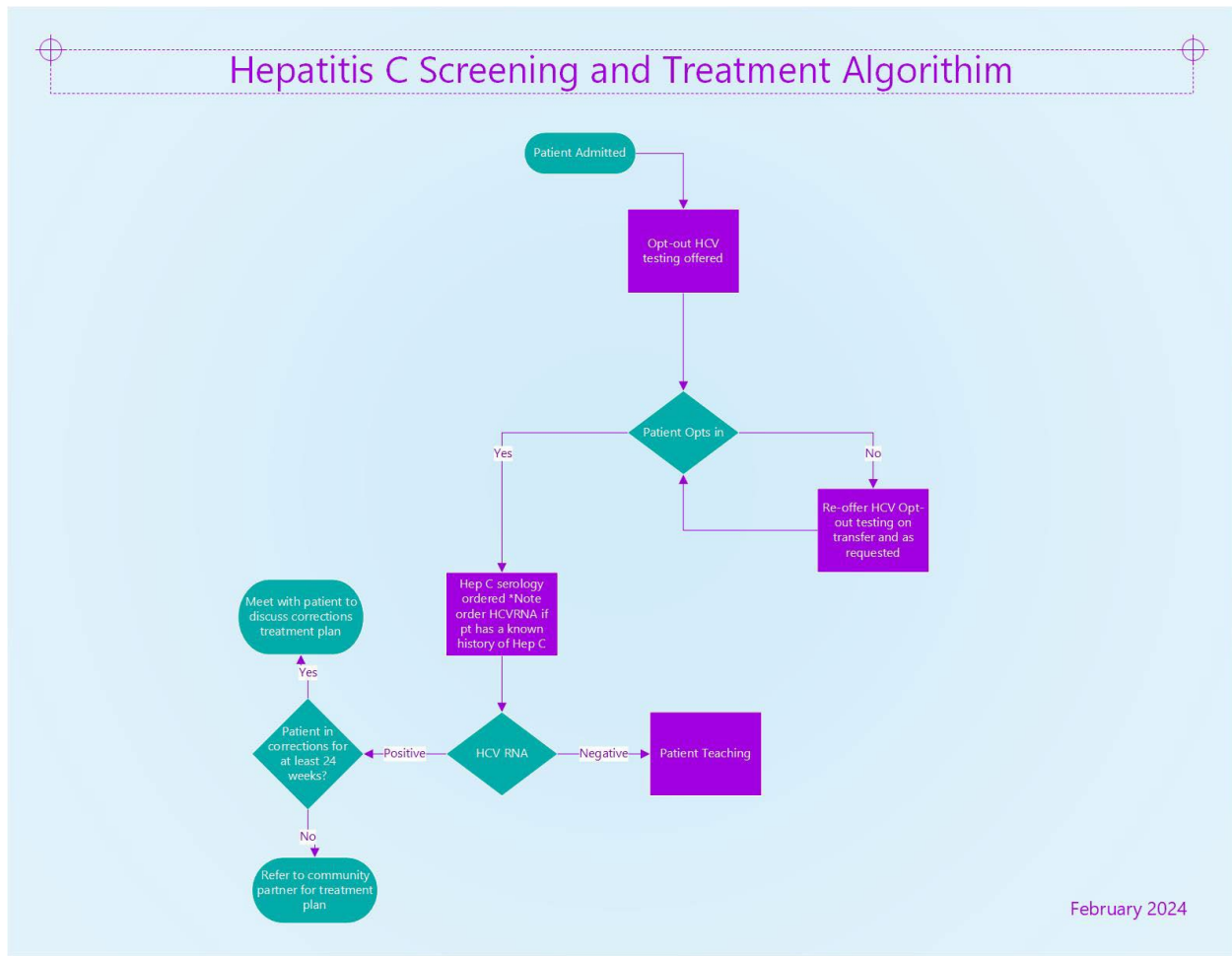
Contact

For more information, please contact:

[Kayley Goralczyk](#)

Provincial Assistant Head Nurse
Correctional Health Services

Appendix A: Hepatitis C Algorithm



Appendix B: Hepatitis C Education and Patient Teaching Script

Education/Disease Overview

Hepatitis C is an infection in the liver caused by the Hepatitis C virus. Over time, the virus causes liver injury/scarring which can make you very sick. You can have Hep C and not know it- it is important to get tested as you often have no symptoms. Symptoms may not occur, if they do they can include: flu symptoms, pain to right side, itchy skin, yellow eyes/skin, brain fog, feeling tired, swelling to legs/abdomen.

Healthy Liver Habits

Your liver helps to get rid of toxins in your body, store fuel/vitamins/minerals, fight infections and helps change food to fuel. Drinking too much alcohol or living with HIV can speed up how quickly your liver is damaged. Hepatitis C can scar your liver and lead to fibrosis. Over time too much scarring can lead to cirrhosis which makes it hard for your liver to function properly.

Transmission

It is passed blood to blood. It gets into the blood through breaks in the skin or through the lining of the nose/mouth. It is a very strong virus and can live in a tiny drop of blood that is too small to see and live outside the body for many days (ex. In a syringe). Dried blood can also pass the virus.

How it can be spread:

- sharing drug equipment- not just injecting also smoking and snorting
- re-using unclean tattoo/ piercing equipment/medical equipment
- sharing personal hygiene equipment (razors/toothbrushes/nail clippers)
- less common to be passed sexually

You can prevent the spread by using your own clean equipment every time and using protection with sex. Hepatitis C can NOT be passed through food/water, dishes, kissing, coughing, toilet seats or swimming pools

Medication

About 1 in 5 people clear Hep C on their own. Treatment is usually needed and comes in pill form. You either take one pill a day for 12 weeks, or 3 pills a day for 8 weeks. The pills are effective 95% of the time. The medication is very expensive- there are a variety of coverage options available. If you will be incarcerated for 24 weeks, the cost is covered by AHS. Sometimes, we can arrange for treatment through community providers. There are very few mild side effects. There is no vaccine for Hep C.

Can I be re-infected?

Yes! Even if you spontaneously clear the virus you can get it again. This is why it is important to avoid high risk behaviours.

Treatment Plans

It takes two blood tests to tell if you have Hep C. One test screens for the antibody to see if you have ever had Hep C. The second test is a viral load test- it tells you if you have Hep C right now. We do both tests from the same tube of blood. Once you are finished the medication, we need to take another blood sample 12 weeks later to make sure the medication worked and to test your liver function.

Appendix C: Hepatitis C Treatment Options

Have they been treated before?

Successfully treated HCV patients do not need a routine HCVRNA ordered. An HCVRNA can be ordered if the patient feels they are at risk for reinfection due to high risk activities or if the patient never completed their 12 week post treatment follow up HCVRNA serology.

When is their next court date/ release date?

The patient's expected length of stay greatly influences their treatment plan.

Does the patient give informed consent to start treatment?

All patients must give informed consent to start any medication. As healthcare professionals, it can be frustrating when a patient declines to start treatment for HCV. The healthcare professional shall provide education and the option to follow up with the patient if the patient changes their mind.

Has the patient ever been connected in the community for HCV treatment?

It has been identified that patients have multiple concerns on release and sometimes HCV treatment and follow up is low on their priority list. If a patient has already built a relationship with a community partner, there is a greater chance that the patient will follow up with them on release. Also, the community provider may already have information required on the patient to arrange for treatment through the community. Any previously established community connections should be the first line of follow up.

Does the patient have any kind of drug coverage in the community? (NIHB, AISH, etc.)

Community providers cover HCV treatment through insurance. It is important to know this information for community follow up.

Note: AISH and AB Works are discontinued while a patient is in custody while NIHB is not.

Appendix D: Specific Referral Information

Specific referral information can be found on the Correctional Health Services [SharePoint](#) page under “shared resources”.